

GZ: 2022-0.777.583 dated November 15, 2022 (Procedure Number: DSB-D124.5167)

[Note from the editor: Names and companies, legal forms and product designations, addresses (including URLs, IP and email addresses), file numbers (and the like), etc., as well as their initials and abbreviations may have been abbreviated and/or altered for pseudonymization reasons. Obvious spelling, grammar, and punctuation errors have been corrected.]

## DECISION STATEMENT

The Data Protection Authority decides on the data protection complaint of Wolfgang A\*\*\* (complainant), represented by B\*\*\* & Partner Rechtsanwälte GbR dated October 18, 2021, amended on October 28, 2021 against Dr. med. Erik N\*\*\* (respondent), represented by C\*\*\* & Partner Rechtsanwälte GmbH regarding violation of the right to confidentiality as follows:

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The complaint is dismissed as unfounded.

Legal bases: Art. 51 para. 1, Art. 57 para. 1 lit. f, and Art. 77 para. 1 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter: GDPR), OJ No. L 119 of May 4, 2016, p. 1; §§ 1, 18 para. 1, as well as 24 para. 1 and para. 5 of the Data Protection Act (DSG), BGBl. I No. 165/1999 as amended; §§ 2 no. 10 lit. a, 13 para. 2, 14 para. 2, 20a, 24c, 24f, 27 para. 14b of the Health Telematics Act 2012 (GTelG 2012), BGBl. I No. 111/2012 as amended by BGBl. I No. 34/2021.

## REASONING

### A. Submissions of the parties and procedural history

1. The complainant stated in his initiating complaint dated October 18, 2021, amended on October 28, 2021, that the respondent had violated his right to confidentiality by conducting a query of the complainant's e-medication data on February 28, 2021 (sic!) at 13:53, although the respondent had never been his treating physician. Attached was an ELGA query protocol of the complainant, which indicated a call of the e-medication data on February 18, 2021, at 13:53:25 by the respondent.
2. The respondent then summarized in his statement dated November 16, 2021, that Ms. Olivia A\*\*\* had come to the respondent's practice on February 18, 2021, with her minor son Tobias A\*\*\*, who is also the son of the complainant and is co-insured with both parents, to have him examined and treated. When inserting the e-card of a minor, not only the data of the minor themselves are displayed on the screen, but also the names of those persons with whom the minor is co-insured. To ensure correct billing with the respective health insurance provider, one of these main insured persons must be clicked. In this case, their file is automatically opened by the software, and depending on the settings, either the treatment or master data sheet is displayed. An inquiry with the software manufacturer revealed the following: if the respondent or his assistant had mistakenly clicked on the complainant, a new file would have been generated, and the master data sheet displayed on the screen would have shown this person along with their date of birth and social security number. This data is stored on the minor's e-card; no further information is visible. However, a connection to ELGA had already been established, and therefore the ELGA protocol noted that the e-medication data had been called up. To actually do this, an additional "click" would have been required, which definitely did not occur, as can also be seen from the e-medication data retrieval protocol of the respondent's practice. The respondent does not know the complainant and had no reason to access his e-medication data. Rather, the respondent's assistant likely clicked on the complainant by mistake and then realized that he was not registered as a patient in the practice, as not even his address data was displayed, and therefore closed the automatically generated master data sheet. Before the COVID-19 pandemic, e-medication data could generally only be accessed if either the e-card of the relevant person or the practice card of the treating physician was inserted; however, this was temporarily changed due to the pandemic by amending the Health Telematics Act until March 31, 2022, to allow for medical consultations via telephone. As a result, it became possible to establish an e-card connection simply by clicking on the main insured person, leading to the note "e-medication data called up."
3. The complainant responded in his statement dated December 3, 2021, that the respondent's explanations were not comprehensible, particularly as it could not be understood that, according to the

query protocol submitted by the respondent himself, a connection had already been established when inserting the e-card, and thus already at 11:26 and in any case no later than at 12:15, and not only at 13:53. If a connection to ELGA could be established simply by clicking on the main insured person without inserting the e-card, and subsequently led to the note "e-medication data... called" would thus render the system of traceability of accesses absurd.

4. The respondent countered in a further statement dated December 16, 2021, that February 18, 2021, was a Thursday, on which day the respondent's practice was only open in the morning; however, the assistants regularly check all patient files of the patients treated that day after the practice closes and make any necessary corrections. In the context of a further inquiry with the software manufacturer, it was now possible to ascertain how the divergence in the access times could have occurred: all patients are listed only once in the access log, and the time from the first opening to the last closing is recorded. This was the case here from 11:26 AM to 12:15 PM. During this time, the complainant, as the main insured person, was not clicked on. It was likely that when processing the patient files in the afternoon, specifically at 1:53 PM, the respondent's master data sheet was mistakenly clicked, after which the note was made.

5. The complainant replied in his statements dated January 11, 2022, and January 27, 2022, summarizing that the respondent again stated that the access log records the time from the first opening to the last closing; however, if the last closing occurred at 12:15 PM, this still would not explain how the note recorded on February 18, 2021, at 1:53 PM came about. Furthermore, the complainant has now also had to ascertain that the respondent accessed his e-vaccination passport data and e-medication data on November 2, 2021, and November 11, 2021.

6. The respondent explained in his statement dated February 18, 2022, that for a patient who is already registered, the patient file is opened when the e-card is inserted; this would consist of a master sheet, treatment sheet, status sheet, laboratory sheet, and findings sheet. When the treatment sheet is opened, a connection to the ELGA is established, without displaying any stored data there; however, to actually open this, an additional click on the "Medication List" button is required. The same applies to the status sheet, through which access to vaccination data can also be obtained subsequently. Both the note "e-medication data accessed" and the note "vaccination passport data accessed" are thus made as soon as the treatment sheet or the status sheet has been opened, but this has nothing to do with an actual review of these data. Regarding the note from February 18, 2021, it was essentially stated as before. Regarding the note from November 2, 2021, the respondent stated that he had received the letter from the ELGA Ombudsman's office dated October 27, 2021, a few days prior, which indicated that an inspection had taken place on February 18, 2021.

To understand this incident, he opened the automatically generated patient file from February 18, 2021. It was noted that only the main sheet was visible, and thus only the name, social security number, and date of birth of the complainant were visible; no further access to the e-medication data had occurred. The note dated November 11, 2021, was made because the respondent had scheduled an appointment with his legal representative on that day in order to provide the statement requested by the Data Protection Authority in a letter dated November 2, 2021. During this meeting, the functionality of the software was also explained, and at that time, the respondent was not yet aware that due to the amendment of the Health Telematics Act, even opening the patient file would result in a note being made. This was only clarified through extensive discussions with the software company X\*\*\* GmbH. At no time did they access the vaccination data or the e-medication data of the complainant.

7. In response to an inquiry from the Data Protection Authority, ELGA GmbH stated in its letter dated July 21, 2022, that it had no jurisdiction over the e-medication data; this jurisdiction lies with the respective ELGA-GDA or the ELGA system partners according to §§ 20 and 22 of the Health Telematics Act 2012 (GTeIG). The Central Vaccination Register is technically implemented in such a way that accesses to it must always be logged (§ 24f Abs. 5 GTeIG). Since ELGA GmbH does not have the relevant log excerpt and cannot technically access the log, it cannot provide any reason for the entry of a note; however, the presented explanation variant of the ELGA-GDA (Note: health service provider, referring here to the respondent) appears to be understandable. Accessing and retrieving are different steps in the technical implementation of the connection of ELGA-GDA to ELGA. Therefore, it

may be that a log entry is made without the ELGA-GDA having "seen" the ELGA data. Since ELGA GmbH has no information regarding the medical practice, medical software, and specific handling in the specific practice, it refers to the ELGA-GDA or the manufacturer of the medical software in this regard.

8. X\*\*\* GmbH summarized in response to an inquiry from the Data Protection Authority in its letters dated August 25, 2022 (received on August 29, 2022) and September 19, 2022, as follows: during the relevant period (February 18, 2021, to 2021), a note was made for a main insured person by opening the patient file – treatment sheet area, indicating that his e-medication data had been retrieved, and by opening the patient file – status/vaccination module area, a note was made indicating that e-vaccination data had been retrieved. Regarding the question of whether such a note could have been made for the main insured person as soon as the patient data of a co-insured person was accessed (even without a specific indication)...

the e-Card) It was stated that, in principle, whenever ELGA data is accessed, only that of the actively opened patient file can be displayed. However, it is possible to switch from the dependent insured's file to the main insured's file via the relative management in the main file. Here, the main insured's file would have been accessed, often due to routine checks for billing purposes, which is a typical procedure in medical practices.

Regarding the question of whether such a note could have been made in the main insured's file without actually accessing the e-medication data or e-vaccination data of the main insured, it was stated that the implementation of the software for accessing ELGA data always occurred under the premise of access authorization via the insertion of the patient's e-Card according to the original specification (data protection regulation). This requirement was suspended due to the COVID situation, and only proof as GDA, through the insertion of the admin card, was necessary. During the requested periods, this regulation was active, which is why access to ELGA data was possible even without the patient's e-Card.

Due to loading times from the ELGA database, there were sometimes delays, so the software was programmed to download ELGA data in the respective area in the background without it being immediately viewable. If access to the data was required, an additional mouse click on the respective button (e.g., ELGA medication list) was necessary. Therefore, a note that ELGA data was retrieved could also have been made without actually accessing this data in the main insured's file. The reason why the access log does not show the call at 13:26 can no longer be reconstructed; however, it has occurred that PC workstations were not regularly shut down or were in standby mode or partially in continuous operation, which could have led to the access log not being recorded correctly.

9. The statements of ELGA GmbH and X\*\*\* GmbH were transmitted to the parties involved in the proceedings by letter from the Data Protection Authority dated September 21, 2022.

10. The respondent subsequently stated in his letter dated September 29, 2022, that he sees his previous assertions confirmed by the information from X\*\*\* GmbH to the extent that ELGA data is downloaded in the background without being immediately viewable, and that accessing and retrieving in the technical implementation are different steps. Finally, the respondent also explicitly pointed out again that the complainant was not a patient in his practice, that he did not even know the complainant, and therefore there was no reason to access the medication data of an unknown person.

11. The complainant stated in his response dated October 31, 2022, that according to the statements of X\*\*\* GmbH and ELGA GmbH, it is theoretically possible that the respondent accessed the complainant's data "without actually having accessed this data in the main insured's file"; however, this is only a possibility, and it cannot be said that no access actually took place. It must be assumed that the respondent unlawfully accessed the e-medication data or e-vaccination data of the complainant multiple times. The statements of the respondent are incorrect and contradictory; the complaint was filed on October 28, 2021, but the respondent stated regarding the note from November 2, 2021, that the email from the ELGA Ombudsman dated October 27, 2021, informing him about the note from February 18, 2021, was first delivered to him on or a few days before that date, making it possible to

trace how this note came about and how the notes from November 2, 2021, came to be. However, these statements are incorrect according to the respondent's own assertions, as he states in his response dated November 16, 2021, that the complaint was only delivered to him on November 10, 2021. Thus, according to his own statements, the respondent had not yet received the complaint on November 2, 2021, which makes the claims about how the notes from November 2, 2021, came to be evidently incorrect. Furthermore, it is noted that the ex-wife of the complainant regularly visited the respondent's practice with their children, and at that time, several court proceedings (divorce proceedings, division proceedings, possession disturbance proceedings, maintenance proceedings, criminal proceedings) were pending between the complainant and his ex-wife, so one can only speculate about what possible medication data could be useful in these proceedings.

## B. Subject of the Complaint

The subject of the complaint is the question of whether the respondent violated the complainant's right to confidentiality by accessing

- the e-medication data on February 18, 2021, at 13:53
  - the e-vaccination data and e-medication data on November 2, 2021, at 9:52
  - the e-medication data on November 11, 2021, at 18:24
- of the complainant.

## C. Findings of Facts

1. The minor son of the complainant was in the respondent's practice for examination and treatment with his mother on February 18, 2021.

2. The minor son of the appellant is co-insured with the appellant and his mother.

Assessment of Evidence: The findings made in C.1. and C.2. are based on the undisputed statements of the respondent in his submission dated November 16, 2021.

3. The medical software used by the respondent was acquired from X\*\*\* GmbH.

Assessment of Evidence: The finding made in C.3. is based on the consistent statements of the respondent and X\*\*\* GmbH.

4. When the e-card of a co-insured minor is scanned, not only the data of the minor themselves is displayed on the screen, but also the names of those persons with whom the minor is co-insured.

5. In order to carry out a correct billing with the respective social insurance carrier, one of the main insured persons must be selected. In this case, their file is automatically opened by the software and, depending on the settings, either the treatment or master data sheet is displayed. (Formatting not reproduced 1:1)

[Note by the editor: the graphic file (Fig. 1 Patient Master Sheet) has been removed as it cannot be pseudonymized in the RIS.]

6. Once a patient file has been automatically generated, access to the treatment and master data sheet could also be made during the contested period between February 18, 2021, and November 11, 2021, without re-scanning the e-card.

7. If a main insured person is selected and their treatment sheet is clicked, a note is made in the ELGA protocol of the main insured person that the e-medication data has been accessed, even if this does not result in actual insight into the e-medication data. Displaying the e-medication data requires an additional click on the "Medication List" button. (Formatting not reproduced 1:1)

[Note by the editor: the graphic file (Fig. 2 Treatment Sheet) has been removed as it cannot be pseudonymized in the RIS.]

8. If a main insured person is selected and their status sheet is clicked, a note is made in the ELGA protocol of the main insured person that the e-vaccination data has been accessed, even if this does not result in actual insight into the e-vaccination data. Displaying the e-vaccination data requires an additional click on the "Vaccinations" button. (Formatting not reproduced 1:1)

[Note by the editor: the graphic file (Fig. 3 Status Sheet) has been removed as it cannot be pseudonymized in the RIS.]

Assessment of Evidence: The findings made in C.4. to C.8. are based on the statements of the respondent in his submission dated February 18, 2022, as well as the documents submitted with it and the consistent statements of X\*\*\* GmbH dated September 19, 2022.

9. According to the ELGA protocol, the e-medication data of the appellant was accessed by the respondent on February 18, 2021, at 13:53 (Formatting not reproduced 1:1).

[Note by the editor: the graphic file (ELGA protocol entry from February 18, 2021) has been removed as it cannot be pseudonymized in the RIS.]

Assessment of Evidence: The finding made in C.9. is based on the statements of the appellant in his initiating complaint and the documents submitted with it.

10. According to the ELGA protocol, in addition, on November 2, 2021, at 9:52, the e-vaccination data and the e-medication data, and on November 11, 2021, at 18:24, the e-medication data of the appellant were accessed by the respondent (Formatting not reproduced 1:1).

[Note by the editor: the graphic file (ELGA protocol entries from November 11, 2021, November 2, 2021, and February 18, 2021) has been removed as it cannot be pseudonymized in the RIS.]

**\*\*Assessment of Evidence:\*\*** The finding made in C.10. is based on the undisputed submissions of the complainant in his statement dated January 27, 2021.

11. The note dated February 18, 2021, was made due to the examination and treatment of the complainant's son, who is also insured under the same policy. In the course of querying the patient file of the minor son of the complainant, the basic data of the complainant, including his name, date of birth, and social security number, were also retrieved.

12. The respondent was informed of the alleged access on February 18, 2021, by a letter from the ELGA Ombudsman dated October 27, 2021.

13. The notes dated November 2, 2021, and November 11, 2021, were made by the respondent during the investigation and reconstruction of the case in question.

14. The respondent did not access the e-medication data or the e-vaccination passport data.

**\*\*Assessment of Evidence:\*\*** The findings made in C.11. to C.14. are based on the statement of the respondent dated February 18, 2022. The Data Protection Authority finds the submissions to be comprehensible, as such a note is made for the main insured complainant— as confirmed by X\*\*\* GmbH— whenever the patient file of the insured minor is accessed. Furthermore, there are no indications that the respondent should have actually accessed this data of the complainant or that would suggest that he did so. It also appears reasonable that the respondent attempted to reconstruct the case first brought to him by the ELGA Ombudsman in the letter dated October 27, 2021, and subsequently transmitted by the Data Protection Authority, and therefore, even without accessing the e-card of the complainant's insured son or an actual appointment, the notes dated November 2, 2021, and November 11, 2021, were made without any actual access to the e-medication data or the e-vaccination passport data of the complainant.

**\*\*D. From a legal perspective, it follows that:\*\***

**\*\*General\*\***

According to § 1 para. 1 DSG, everyone has the right to the confidentiality of their personal data, provided there is a legitimate interest in doing so. The existence of such an interest is excluded if data is not accessible to a confidentiality claim due to its general availability or because it cannot be traced back to the affected individual.

The GDPR and, in particular, the principles enshrined therein must be considered in the interpretation of the right to confidentiality (see the decision of the DSB dated October 31, 2018, GZ DSB-D123.076/0003-DSB/2018).

Based on the complaint submissions, it must be examined whether the respondent unlawfully accessed the e-medication data and e-vaccination passport data of the complainant. Since this data undoubtedly relates to the complainant, the scope of application of § 1 para. 1 DSG is opened.

The respondent is an ELGA health service provider according to § 2 no. 10 lit. a of the Health Telematics Act 2012 (GTelG).

According to § 13 para. 2 in conjunction with § 14 para. 2 GTelG, ELGA health service providers

(ELGA-GDA) have the right to store ELGA health data in ELGA for the treatment or care of ELGA participants and to collect it in accordance with their respective professional duties. ELGA health data also includes e-medication data according to § 20a GTelG.

The data processed within the framework of the eHealth application "e-vaccination passport" involves electronic processing of data from the central vaccination register, which the respondent can access according to § 24f para. 4 no. 1 in conjunction with § 24c para. 2 no. 1 GTelG.

According to § 24f para. 5 GTelG, every access must be logged.

In principle, identity must be verified before querying ELGA data, which is usually done by inserting the e-card. However, during the Covid-19 pandemic, several amendments to the GTelG occurred, allowing for identity verification based on name and social security number even without inserting the e-card, as provided in § 27 para. 14b of the version BGBl. I No. 34/2021.

**\*\*In the matter\*\***

The minor son of the complainant, who was insured under the complainant, was treated by the respondent on February 18, 2021. As credibly presented by the respondent, it appears that the main insured complainant was mistakenly clicked, although the accompanying (also main insured) mother of the minor patient should have been selected instead. As clarified by the established facts, a note was already made in the ELGA protocol due to this, even though there was no actual access to the e-medication data.

The arguments presented by the respondent appear equally understandable, stating that the additional notes from November 2, 2021, and November 11, 2021, were made in the context of the investigation and reconstruction of the case at hand. It is not evident why the complainant assumes that this assertion is internally contradictory and therefore incorrect. The respondent has explicitly stated that he first became active in investigating the case on November 2, 2021, based on the email from the ELGA Ombudsman dated October 27, 2021. Since, according to the respondent's own statements, the relevant complaint was delivered to him by the Data Protection Authority on November 10, 2021, it is equally understandable that he reopened the automatically generated patient file both on November 2, 2021, after receiving the letter from the ELGA Ombudsman, and on November 11, 2021, after receiving the request for a statement from the Data Protection Authority for the reconstruction of the case, resulting in the notes being made, without there being any actual access to the e-medication data or e-vaccination passport data.

To the extent that the complainant asserts that the respondent presumably accessed his health data due to pending court proceedings between the complainant and his ex-wife, this is, as the complainant himself admits, merely a speculation. A general assertion based solely on mere assumptions amounts to an inadmissible exploratory evidence, which an authority is not obliged to consider (cf. the ruling of the Administrative Court of Austria dated January 3, 2018, Ra 2017/11/0207, Rs. 3). It has not even been claimed that the ex-wife of the complainant actually submitted health data of the complainant obtained through the respondent in any of these proceedings.

Furthermore, there are no other indications that there was indeed access to the e-medication data or e-vaccination passport data of the complainant. Therefore, the Data Protection Authority has concluded in its assessment of the evidence that such access to the e-medication data or e-vaccination passport data did not occur.

The complaint was therefore to be dismissed as unfounded.